

COMMUNICATION BARRIERS AND CHALLENGES IN HEALTH EDUCATION ABOUT COVID-19 IN INDIGENOUS COMMUNITIES OF THE HUASTECA POTOSINA.

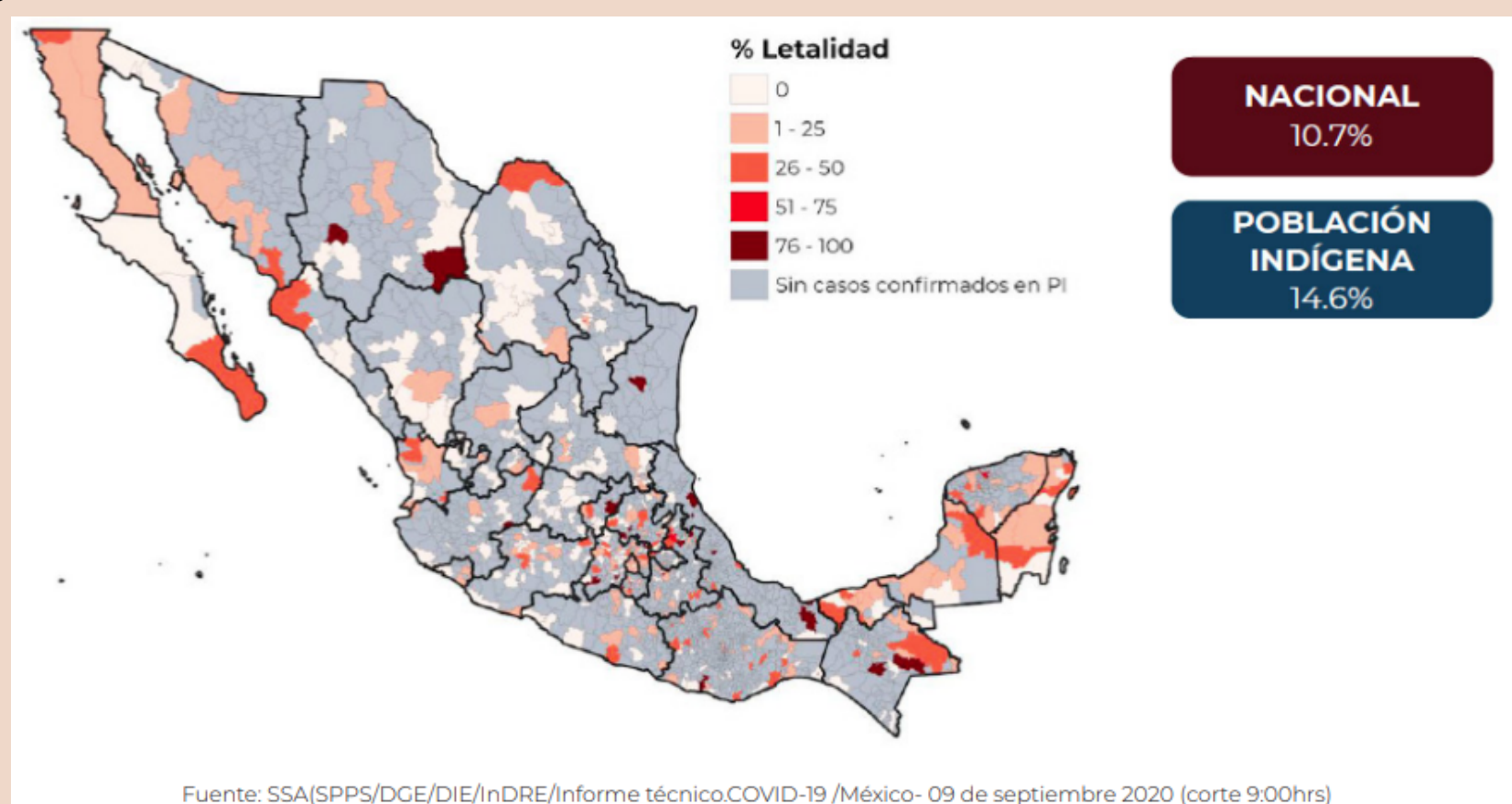
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INTRODUCTION

The COVID-19 pandemic has had a significant global impact. In Mexico, the most affected sector has been the indigenous population, because the government and the national health system addressed this public health problem without the existence of a health guideline or policy aimed at indigenous peoples. Most of the actions were taken when the contagion had spread, weakening their ability to prevent and manage the pandemic.

JUSTIFICATION (EVIDENCE OF VULNERABILITY IN INDIGENOUS PEOPLES)

At the beginning of the pandemic, the National Council for the Evaluation of Social Development Policy [CONEVAL] (2020) evaluated 19 priority programs to address the health crisis, focused on the most vulnerable sectors. According to this evaluation, none of the programs were specifically aimed at the populations living in poverty. In 2020, 29.2% of the indigenous population was in extreme poverty, 77.2% did not have social security while the lack of access to health services increased from 15.3% to 32.8% during this year (CONEVAL, 2020). Furthermore, the vast majority did not have access to basic services in housing and food and as a consequence the fatality rate in the indigenous population was 14.6%, which is higher than the national average, which was 10.7%.



Map 1. Distribution of the COVID-19 mortality rate in the indigenous population in 2020.

MYTHS ABOUT COVID-19

During the pandemic, false information on COVID-19 was circulated on media and social media. For example, that doctors extracted synovial fluid from the knee of hospitalized COVID patients for later sale according to Fernández (2023). Another of the accepted theories was that it was a strategy by the government to reduce the population with some comorbidity (Diabetes, Hypertension, Dyslipidemia, Cancer) to reduce treatment costs (Mansilla, 2021), so many people resorted to remedies. homemade products and traditional medicine of indigenous peoples.

This misinformation generated collective resistance to health measures, developing a generalized fear, avoiding going to health institutions due to the belief that the virus was there, or for fear of being isolated if they showed any symptoms. Furthermore, the information disseminated focused only on preventive measures; there was little information about the disease process. One of the weak points was the information disseminated through listening material in the mother tongue because this was only available on an indigenous radio station. Currently few people tune into this radio station. The available visual material did not have a great impact because it was not considered that 30.6% of adults aged 75 years and over in San Luis Potosí do not know how to read and write (INEGI, 2020).

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RESEARCH QUESTION

This is why the present investigation aims to provide materials and data that can remedy the lack of health prevention knowledge and also answer the following question:

How can we mitigate the effect of lack of knowledge and disinformation around COVID-19 on the health of the indigenous peoples of the Huasteca Potosina?

POSSIBLE INTERVENTIONS

TO ANSWER THIS RESEARCH QUESTION, I PROPOSE A MIXED STRATEGY:

Development of educational material aligned with community needs, for example:

- Make a diagnosis to determine the level of knowledge about the disease.
- Identify which information is useful and important to them and in which areas the information provided is not clear.
- Develop communication channels at the individual, family and local government level, based on the culture and language of the audience with respect to their worldview.
- Develop workshops in the mother tongue based on understandable messages aimed at key people or the person who has the greatest influence in the community and who in turn this person, can encourage active and conscious participation in the community and thus achieve self-care of health.
- Collaborate with indigenous representative organizations to generate greater impact.

Quantitative evaluation of material effectiveness in communities:

- Experimental or quasi-experimental methodologies to evaluate the causal effect of the previously developed material. This includes the development of a survey and knowledge measurement tools, to be collected pre and post intervention.
- Mapping and analysis of community social networks; a common methodology in the area of epidemiology.

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